WACO FAMILY MEDICINE

Business Plan for Clinical Pharmacy Service

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Executive Summary

This business plan was created to secure commitment and funding to open a clinical pharmacist position as part of a multidisciplinary team care at Waco Family Medicine (WFM) to serve the underserved residents McLennan and Bell counties.

Background

The clinical pharmacy service will be offered to the primary care providers at Waco Family Medicine – Central location. The clinical pharmacist will be positioned within the clinic for easy recognition and patient accessibility. Clinical pharmacy staff with advanced training in ambulatory care, will provide chronic disease management services to patients referred by their primary care physician as well as addressing medication consults and maximizing reimbursement metrics. A clinical pharmacy service will augment the organization's goal of increasing access to high quality, comprehensive primary, and preventative health care to the underserved patients of McLennan and Bell counties.

The Market

Based on most recent data, Texas ranks 41st in the nation for physician-to-population ratio. For primary care, Texas ranks in at 47th. Although pharmacists are not recognized as providers federally, pharmacists are becoming integral members of interdisciplinary care teams in many health systems. By utilizing collaborative practice agreements, pharmacists may assist in meeting organizational goals by relieving some physician burdens such as medication reconciliation, medication access (prior authorizations, insurance formulary constraints), transitions of care, medication management, monitoring parameters, and optimizing reimbursement metrics through appropriate billing codes

Organization Structure

The clinical pharmacist will report to the director of pharmacy and the medical director of Waco Family Medicine. The director of pharmacy is responsible for the organization's pharmacy operations and the medical director is integral in the clinical management of ambulatory patients.

Finances

The major expense to establish the service includes hiring an appropriately trained clinical pharmacist. Other expenses will include facility/office space, clerical staff to assist with scheduling, and medical/office supplies as needed. Some expenses may be offset by co-funding the clinical pharmacist position with a college of pharmacy.

Conclusion

The addition of a clinical pharmacist at Waco Family medicine will augment current provider services, improve access to medication management services, and promote optimal drug therapy for the patients of McLennan and Bell countries.

Clinical Pharmacy Service Proposal

I. Purpose

To establish a clinical pharmacy service where a pharmacist will work in collaboration with Waco Family Medicine providers to provide chronic disease state management with all aspects of medication selection, access, management, monitoring, and patient education.

II. Economic Impact

With the evolving nature of health care, health systems have undergone dramatic changes to improve quality of care and outcomes. Many institutions are now focused on the "quadruple aim" of enhancing patient experience, improving population health, reducing costs, and improving work-life balance of healthcare providers.[1] Clinical pharmacists are in a unique position to be an essential member of the collaborative team to target the quadruple aim.[2] Pharmacists are among one of the most accessible and underutilized health care professionals in the field. According to a report by the Centers for Disease Control and Prevention (CDC), pharmacists can reduce fragmentation of care, lower health care costs, and improve health outcomes.[3-4]

One way for pharmacists to make an impact is to utilize collaborative drug therapy management (CDTM) enabled by a collaborative practice agreement (CPA) with providers. CDTM allows pharmacists to perform specific patient care functions such as ordering labs, administering immunizations, selecting, initiating, monitoring, continuing, and adjusting drug regimens.[5] CDTM has been shown effective in improving clinical outcomes including lower blood pressure, HgbA1C, and LDL; improving treatment quality through pharmacist compliance of guideline directed therapy, and increasing patient knowledge and adherence to medication regimens.[6]

Research suggests that clinical pharmacy services such as CDTM can be clinically beneficial as well as economically beneficial to the healthcare system.[7] There are case examples such as the Asheville Project, the Patient Self-Management Programs for Diabetes (PSM), and the Diabetes Ten City Challenge (DTCC). These programs were efforts by self-insured employers to provide education and mentoring for employees with chronic health problems. Patients were enrolled in collaborative care programs that included a community pharmacist. The program showed average net savings of $1622-$3356 per person/year for the Asheville Project, $918 per person/year for PSMP, and $1079 per person/year for DTCC. Furthermore, the Asheville project showed more than 50% of patients demonstrated A1C improvement, PSMP showed 94% of patients met the Healthcare Effectiveness Data and Information Set (HEDIS) goal of 7% or less for A1C, and DTCC showed 91% of patients achieved an A1C level that met HEDIS goal.[8-12]

Furthermore, a comprehensive review that included 93 articles from 2001-2005 demonstrated the economic impact of the clinical pharmacy services. The most common types of clinical pharmacy services evaluated were general pharmacotherapeutic monitoring services, target drug programs, and disease state-management services. The return on investment for clinical pharmacy services was $4.81 of savings to every $1.00 invested into employing pharmacists.[13]

III. Organization

Waco Family Medicine (WFM) is a Federally Qualified Health Center (FQHC) providing healthcare to underserved residents of McLennan and Bell counties who have historically struggled in a healthcare system that favors privately insured patients. In 2020, WFM served 59,435 patients, providing integrated medical, dental, and behavioral health care across 14 clinic sites.

IV. Mission Statement

Our mission is to increase access to high quality, comprehensive primary and preventative health care for the vulnerable of the Heart of Texas and to provide an excellent educational, training, and research environment in the medical, dental, and behavioral health fields.

V. Objectives

It is the goal of the service to best utilize a clinical pharmacist to:

- i. Manage chronic disease states
- ii. Improve medication adherence
- iii. Decrease long-term complications (MI, stroke, ESRD etc.)
- iv. Decrease hospitalizations and re-admissions
- v. Serve as drug information resource
- vi. Promote cost saving measures (meeting waiver metrics, formulary management, etc.)
- vii. Promote pharmacy profession by serving as preceptor as an educator for future pharmacists by partnering with the college of pharmacy

VI. Clinical Services Rendered

- i. **Chronic Disease State Management**
  - 1) Diabetes
  - 2) Hyperlipidemia
  - 3) Hypertension
  - 4) Congestive Heart Failure
- ii. **Anticoagulation**
  - 1) Warfarin Management
    - a. Monitoring INR and adjusting therapy for safety and efficacy
    - b. Bridge therapy plans during interruption of warfarin therapy
    - c. Providing patient education for newly initiated patients

2) Direct-Acting Oral Anticoagulants (DOACs)
    a. Monitoring labs every 3-6 months in regard to safety and efficacy
    b. Bridge therapy plans during interruption of DOAC therapy
    c. Providing patient education for newly initiated patients or patients transitioning from warfarin

iii. **General Medication Management**
1) Comprehensive Medication Review (CMR)
2) Post discharge medication review
3) Device teaching (glucometer, insulin pen, etc.)
4) Patient counseling
5) Medication adherence education and pillbox refills
6) Medication access (prescription assistance, prior authorizations, etc.)

iv. **Drug information resource**
1) Serve as pharmacy resource for providers in clinic

VII. Quality Metrics

In order to measure quality of care, there are sets of quality measures used to determine payment to hospitals, physicians, and other healthcare providers.

i. **Merit-Based Incentive Payment Systems (MIPS)**
1) Documentation of current medications
2) Medication adherence
3) HgbA1C <9%
4) Use of high-risk medications in elderly
5) Screening for high blood pressure and follow-up
6) Controlling BP (<140/90 mmHg)

ii. **The Healthcare Effectiveness Data and Information Set (HEDIS) Measures**
1) A1C testing
2) A1C < 9%
3) BP control < 140/90 mmHg
4) LDL-C screening and LDL < 130
5) Retinal Eye exam
6) Nephropathy monitoring

iii. **1115 Waiver Metrics** (Texas Medicaid)
1) Diabetic foot exam
2) HgbA1C < 9%
3) DM + BP < 140/90
4) Screening for high blood pressure
5) Statin therapy
6) Tobacco use
7) BMI screening + counseling
8) Immunization status (influenza, pneumococcal, etc.)

Resource: https://www.cdc.gov/diabetes/dsmes-toolkit/business-case/quality-measures.html

VIII. Strategic Implementation

i. Clinical Pharmacist onboarding

1) A clinical pharmacist will be hired to collaborate with providers to meet clinical and economic outcomes.
2) The clinical pharmacist will display competency by maintaining specialty board certifications such as Board Certification in Ambulatory Care (BCACP) or CDE (Certified Diabetes Educator).
3) Split-funding pharmacist salary with Texas A&M College of Pharmacy will help off-set the cost of service as well as help foster the education of future pharmacists. Student pharmacists can serve as care extenders under direct supervision of the preceptor.
4) Time will be split between obligations between WFM and Texas A&M as deemed appropriate by the two organizations.

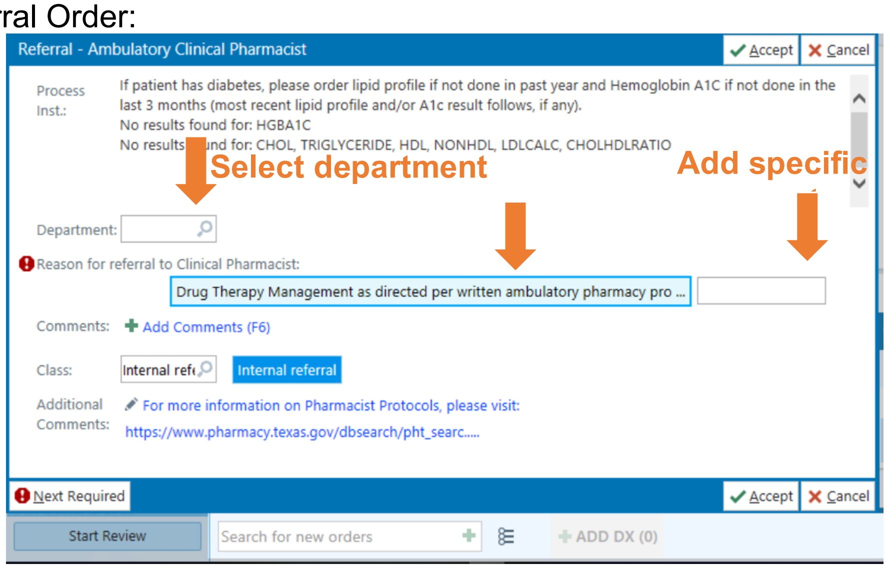
ii. CDTM and CPA

1) CDTM between the pharmacist and providers will be outlined by developing a CPA between the clinical pharmacist and delegating providers.
2) The CPA will outline pharmacist scope of practice and allow the pharmacist working within a defined protocol to perform patient assessments, counseling, referrals, ordering laboratory tests, and selecting, initiating, monitoring, and adjusting medication regimens.
3) The CPA will need to be signed by all delegating providers and approved by the Texas State Board of Pharmacy.

iii. Patient referrals

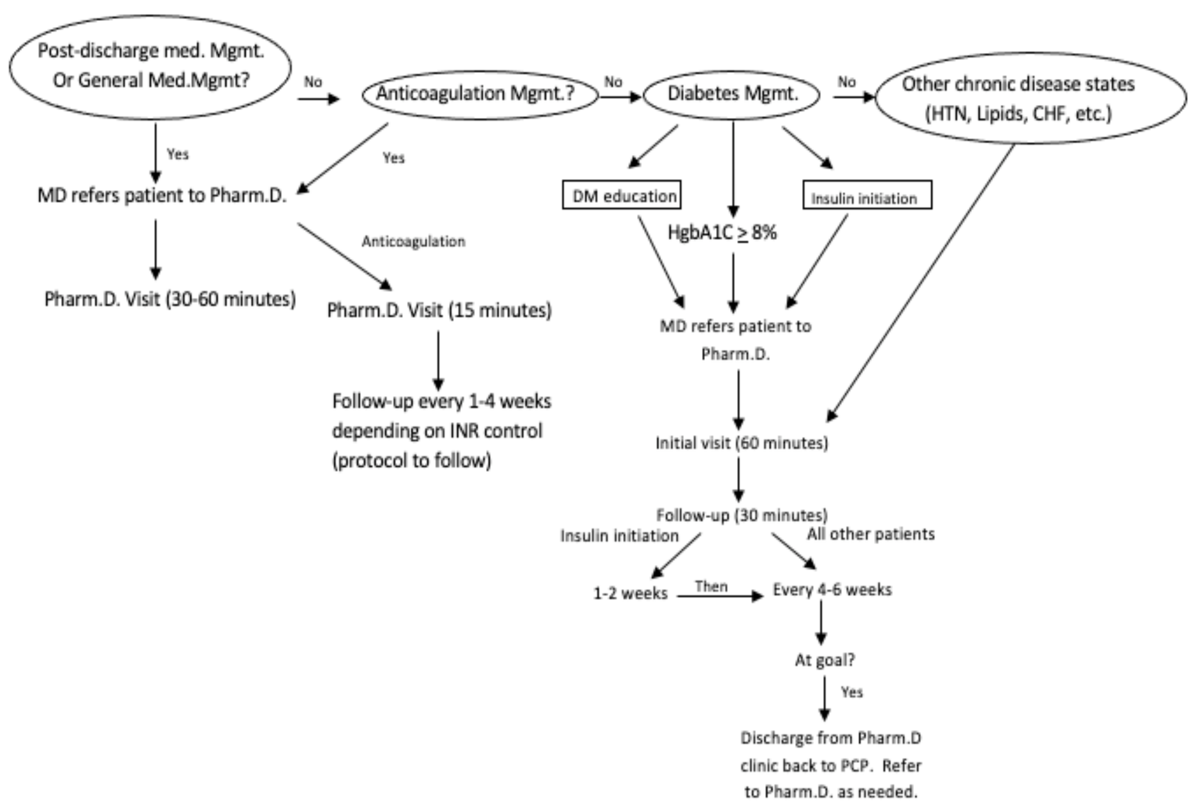
1) Patients will be referred to the clinical pharmacy service by Waco Family Medicine providers. Referrals will be placed as an order in EPIC EMR and sent to the appropriate work que managed by the pharmacist.

Example Referral Order:



2) Referral orders will be used to track and maintain patient panel as well as generate reports. Referrals will be closed once the patient is discharged from pharmacy services.
3) Referrals will be reviewed by clinical pharmacy for appropriateness based on designated criteria and at the discretion of the pharmacist.
4) Once approved, appointments for initial visit to be scheduled by clerical staff or pharmacist.
5) Initial visit will usually be 60 minutes. Follow-up visits will be 15 minutes for anticoagulation and 30 minutes chronic disease management.
6) Once enrolled into the clinic, patient will have regular follow-up visits with pharmacists depending on need (1-6 weeks). Patients must still have regular PCP follow-up at least annually.
7) Criteria for discharge from services
   a. Met goals of therapy and there is no longer a need for medication intervention
   b. Non-compliance at the discretion of the pharmacist
      - Non-compliance (no-shows/lost to follow-up for multiple weeks)
      - Patient request

Example Clinic Referral Flowsheet



iv. **Pharmacist role**

1) The pharmacist will engage in drug therapy management as outlined in CPA. Services include assessment of drug management, collecting patient history, assessing adherence, providing patient education, ordering appropriate labs and medications.
2) Pharmacist will provide care to patients in face-to-face visits, virtual visits, and phone calls as appropriate.
3) Pharmacist will document drug regimen, significant, findings, and services in patient's medical records. The authorizing physicians will have access to clinical pharmacist's documentation in patient's electronic medical records.
4) Pharmacist will provide necessary communication and status reports to supervising physicians as necessary.
5) Pharmacist will also serve as a medication resource for providers as necessary (consults).

v. **Proposed Pharmacist Schedule**

1) Pharmacist will dedicate 3 days for patient care and 2 days for education duties with time allotted for other faculty responsibilities as appropriate during the semester.
2) Patient care days will be for clinic responsibilities such as patient visits and meetings.
3) Education days will be allotted for faculty responsibilities such as precepting or lecturing.

Example Schedule:

	Mon	Tues	Wed	Thurs	Fri
AM	Patient Care 0800 - 1200	Education	Patient Care 0800 - 1200	Education	Patient Care 0800 - 1200
PM	Patient Care 1300 - 1700	Patient Care 1300 - 1700	Patient Care 1300 - 1700	Education	Education

IX. Assessment & Outcomes

i. Using metrics such as HEDIS Measures and 1115 waiver measures, we can measure success of the program as well as maximize reimbursement rates.

ii. We will run EPIC outcomes reports every month to collect data regarding:

1) Last HgbA1c reading
2) Preventative measures: foot exam, eye exam, statin therapy, aspirin for primary/secondary prevention, immunizations etc.
3) Vitals: blood pressure, heart rate, weight, BMI, etc.

4) Laboratory orders: lipid panel, micro-albumin ratio, liver function tests, basic metabolic panel
5) Completed/no-show visit rates

iii. Gap analysis - data after 1 year will be collected and compared to baseline in order measure impact of clinical pharmacy service. Suggested measures may include:
1) % change in HgbA1C, blood pressure, lipid panel, BMI
2) Preventative measures: aspirin, statin therapy, immunization status
3) Number of labs ordered, and prescriptions written by pharmacist
4) Hospitalization and re-admission rates

X. Billing Opportunities

i. **"Incident-to" Billing CPT codes**
1) Evaluation and management of an established patient
2) Since pharmacists are not recognized as providers nationally, can only be billed as level 1.

Code	Time Spent	Estimated reimbursement
99211 (Level 1 minimal)	5 minutes	$23.07

Resource: https://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/eval-mgmt-serv-guideICN006764.pdf

ii. **Medication Therapy Management (MTM) visits**
1) Must be face-to-face and include review of pertinent patient history and recommendations to improve medication outcomes and patient compliance
2) Only billable currently through MTM contract with Medicare Prescription Drug Plan.
3) Reimbursement is set by Part D sponsor

Code	Time Spent	Patient Type
99605	MTM services by pharmacist; 15 minutes	New Patient
99606	MTM services by pharmacist; 15 minutes	Established patient
99607	MTM services provided by pharmacist; Each additional 15 minutes	Established patient

Resource: www.cms.gov/Medicare/Prescription-Drug-Coverage/PrescriptionDrugCovContra/MTM.html

iii. **Diabetes Self-Management Education/Training (DSME/T)**
1) G codes can be used for DSME/T if the program is recognized by ADA or AADE and if the pharmacist is a certified Diabetes Educator (CDE)
2) Medicare covers 10 hours of education the first year and 2 hours each subsequent year.

3) FQHCs with an accredited program can bill for DSMT or MNT services. However only individual services qualify as a separate encounter, so they are able to be billed. Group services do not qualify as billable encounters.

Code	Time Spent	Estimated reimbursement
G0108	Used for each 30 minutes of an individual DSME session	$56.22
G0109	Used for each 30 minutes of a group (2-20 persons)	$15.50 per patient

Resource: For complete information, refer to CMS Benefit Policy Manual: Chapter 15, Section 300. https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-Ioms-Items/Cms012673.html
https://www.myaadenetwork.org/p/bl/et/blogid=160&blogaid=2178

iv. **Transitional Care Management (TCM)**

1) Pharmacists cannot bill but may contribute to the service as a "qualified non-physician provider"
2) Claims must be submitted under a Medicare recognized provider
3) Pharmacists can provide non-face-to-face care coordination component of the visit
4) Pharmacist can be involved in the face-to-face visit and assist provider in medication decision making services

Code	Service	Estimated Reimbursement
99495	Medication decision making of at least moderate complexity. Face-to-face visit within 14 calendar days of discharge	$166.50
99496	Medication decision making of at least high complexity. Face-to-face visit within 7 calendar days of discharge	$234.97

Resource: https://www.aafp.org/dam/AAFP/documents/practice_management/payment/tcm-faq.pdf
Resource: www.cms.gov/apps/physician-fee-schedule

v. **Chronic Care Management (CCM)**

1) Pharmacists cannot bill but may contribute to the service as a "qualified non-physician provider"
2) Applicable to beneficiaries with two or more chronic conditions that are expected to last at least 12 months or until death
3) Patient consent must be obtained annually

Code	Time Spent	Estimated Reimbursement
99490 (Regular or non-complex)	At least 30 minutes of clinical staff time	$42.17
99487 (complex)	60 minutes of clinical staff time with moderate or high complexity medical decision making	$92.98
99489 (complex)	Each additional 30 minutes of clinical staff time	$46.49
G056	Add on-code that can be reported once per CCM billing practitioner, in conjunction with CCM initiation	$63.43

Resource: www.cms.gov/apps/physician-fee-schedule
Resource: https://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/ChronicCareManagement.pdf
Resource: https://www.acponline.org/system/files/documents/running_practice/payment_coding/medicare/chronic_care_management_toolkit.p

vi. **Medicare Annual Wellness Visit (AWVs)**
  1) Pharmacists cannot bill Welcome to Medicare (IPPE) Visit (G0402) as this must be completed by physician within first year of enrollment
  2) Pharmacist cannot bill directly for these visits in an FQHC. The Physician provider must bill for the service after having face-to-face contact with the patient.

Code	Service	Estimated Reimbursement
G0438	First Annual Wellness Visit	$174.43
G0439	Subsequent Annual Wellness Visit	$118.21

Resource: For complete information, refer to CMS Benefit Policy Manual: Chapter 15, Section 280.5. https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Downloads/bp102c15.pdf
Resource: https://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/downloads/AWV_chart_ICN905706.pdf
Resource: www.cms.gov/apps/physician-fee-schedule

XI. Resources Needed:

i. **Pharmacist** (Salary $60/hour)
  1) Split funding between Texas A&M College of Pharmacy and Waco Family Medicine (50/50)

ii. **Clerical staff** (Salary $15/hour)
  1) Aid with scheduling and check-in/check-out

iii. **Facility Space** ($5000-6000)
  1) Office Space + Computer
  2) Patient Care Room

iv. **Medical and office Supplies as needed** ($1750)

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